



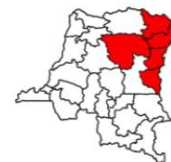
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°103/MVEBDB/08/25/2026

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HIGHLIGHTS (last 24 hours)

As of August 25, 2026, there were 57 new confirmed cases, including 11 community deaths, of Ebola Virus Disease.

Ebola cases (EVs) have been recorded in the provinces of Ituri (34 cases), North Kivu (18 cases), and Haut-Uélé (5). No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 5,713	ACCUMULATIVE DEATH CONFIRMED 2,744	LETHALITY 48.0%	PATIENTS IN ISOLATION/CTE 770	ACCUMULATIVE HEALED 1,269	TRACKING RATES CONTACTS 84.6% (Of the Day)
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo & Bas Uélé



58 out of 151
health zones affected

(38.4%)



Summary of key points from the last 24 hours

In the past 24 hours, **57 new confirmed cases**, including **11 community deaths** from Ebola Virus Disease (EVD), have been reported in the provinces of **Ituri (34 cases)**, **North Kivu (18)**, and **Haut Uélé (5)**.

As of August 25, 2026, **24 patients** were declared cured after obtaining two negative results for Ebola Virus Disease (EVD) (20 in Ituri, 3 in North Kivu and 1 in Haut Uélé).

On the other hand, **18 intra-CTE deaths** were recorded, including 14 in Ituri, 2 in North Kivu and 2 in Haut Uélé.

From the start of the epidemic until August 25, 2026, **5,713 confirmed cases, including 2,744 deaths**, were reported in the DRC, representing a case fatality rate of **48.0 %**. Ituri remains the epicenter of the epidemic, accounting for 83.1% of cumulative confirmed cases and **59.6%** of new cases reported in the last 24 hours.

No new health zones have been affected in the last 24 hours.

In total, 58 of the 151 health zones in the six provinces are now affected since the start of the epidemic in the DRC. Ituri has 28 health zones affected out of 36, followed by North Kivu (13/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (3/11).

Distribution of confirmed cases and deaths by affected province

(As of August 25, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	4750	2,133	44.9%	28/36 (77.8%)	34
North Kivu	753	512	68.0%	13/34 (38.2%)	18
Haut-Uélé	189	89	47.1%	6/13 (46.1%)	5
Tshopo	15	8	53.3%	7/23 (30.4%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Bas Uélé	3	1	33.3%	3/11 (27.3%)	0
Total	5,713	2,744	48.0%	58/151 (38.4%)	57

Confirmed cases and deaths by province and health zone (situation as of August 25, 2026)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	4750	2133	44.9%	34	0	14	14
Adja	11	1	9.1%				0
Aru	7	6	85.7%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	148	29	19.6%	9	0		0
Boga	2	2	100.0%				0
Bunia	1317	411	31.2%	5	0		0
Damascus	27	9	33.3%				0
Drodro	13	6	46.2%				0
Fataki	73	30	41.1%				0
Gethy	5	2	40.0%				0
Kambala	2	0	0.0%				0
Kilo	32	12	37.5%				0
Komanda	61	39	63.9%				0
Lita	204	111	54.4%				0
Logo	11	5	45.5%				0
Lolwa	19	4	21.1%	1	0		0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%	3	0		0
Mandima	32	19	59.4%				0
Mangala	203	109	53.7%				0
Mongbwalu	605	284	46.9%	2	0		0
Nia-Nia	199	108	54.3%	6	0		0
Nizi	628	252	40.1%	6	0		0

Nyankunde	124	35	28.2%	1	0		0
Rimba	10	4	40.0%				0
Rwampara	921	335	36.4%	1	0		0
Tchomia	55	25	45.5%				0
To ventilate	N / A	280	N / A	N / A	N / A	14	14
North Kivu	753	512	68.0%	18	9	2	11
Blessed	118	85	72.0%			1	1
Butembo	146	110	75.3%	6	3	0	3
Goma	1	0	0.0%				0
Kalunguta	16	7	43.8%	1	0	0	0
Katwa	360	241	66.9%	9	4	1	5
Kyondo	16	12	75.0%				0
Lubero	1	1	100.0%				0
Mabalako	3	2	66.7%				0
Masereka	7	4	57.1%				0
Musienene	72	43	59.7%	2	2	0	2
Mutwanga	1	1	100.0%				0
Oicha	7	4	57.1%				0
Vuhovi	5	2	40.0%				0
Haut-Uélé	189	89	47.1%	5	2	2	4
Boma Mangbetu	26	14	53.8%			1	1
Gombari	3	1	33.3%				0
Isiro	74	32	43.2%	4	1	1	2
Pawa	20	15	75.0%				0
Rungu	1	0	0.0%				0
Wamba	65	27	41.5%	1	1	0	1
Tshopo	15	8	53.3%	0	0	0	0
Bafwasende	1	0	0.0%				0
Kabondo	3	1	33.3%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	6	4	66.7%				0
Mangobo	2	2	100.0%				0
Tshopo	1	0	0.0%				
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	3	1	33.3%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				
Ganga	1	0	0.0%				
Total	5713	2744	48.0%	57	11	18	29

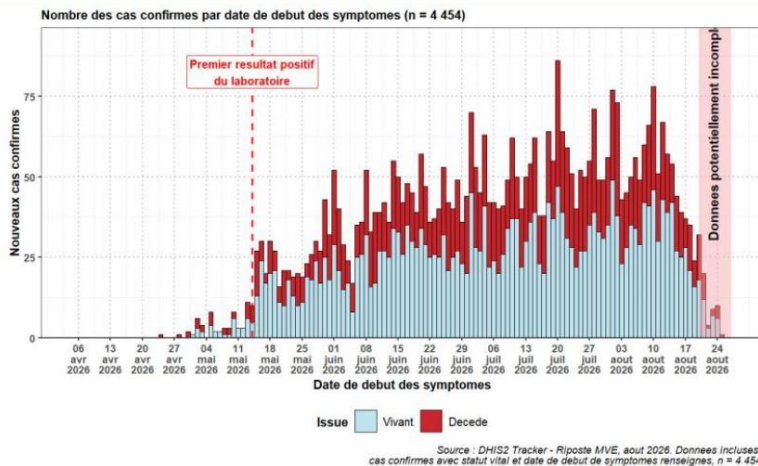
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

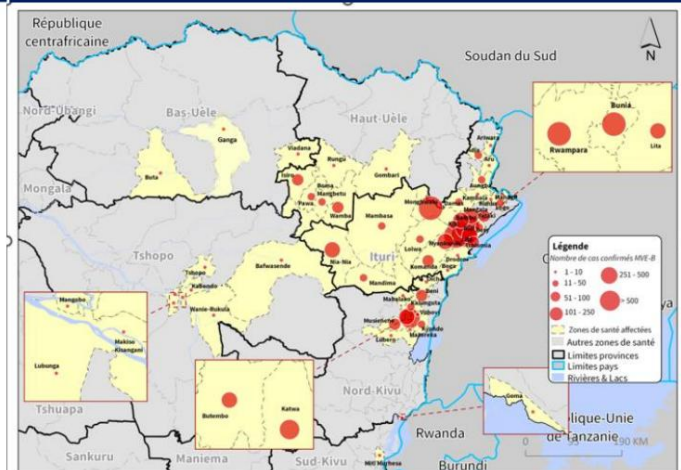
Status of alerts notified by province, as of August 25, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	ND	ND	ND	ND	ND	ND	ND	ND	ND
Bas Uélé	17	2	19	1	2	16	0	3	1
Ituri	840	70	910	164	65	484	0	229	129
North Kivu	1,241	50	1291	95	50	1146	0	145	63
South Kivu	13	0	13	6	0	7	0	6	6
Tshopo	137	1	138	7	0	130	1	7	5
Total General	2248	123	2,371	273	117	1783	1	390	204

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **North Kivu** : Coordination meeting held.
- **Ituri** : Continued supervision of response activities and holding of the meeting of the coordination.
- **Tshopo** : Coordination meeting held and supervision of activities continued of the response.
- **South Kivu** : Continued intensification of surveillance and active search in the Ibanda and Nyangezi health zones; the province remains without new confirmed cases and the last one dates back to June 10, 2026.

1.1.2. Epidemiological surveillance

1.1.3. Main Actions

At the end of the day on August 25, 2026, 2,371 alerts were recorded, including 2,174 (91.7%) were verified in the five provinces that submitted their data. Following these verifications, 390 (17.9%) were validated as suspected cases; all 390 suspects were investigated, i.e., 100%, of whom 204 were transferred to CT/CTE.

The proportion of contacts followed up in the last 24 hours is 84.6% (18,574 seen out of 21,959 to be followed up), below the threshold of 85%: Ituri 87.3% (11,276/12,912), Tshopo 100.0% (489/489) and North Kivu 80.6% (6,672/8,274) and Bas Uélé 48.2% (137/284) show low follow-up rates (data from Haut Uélé province were not reported).

In Bas-Uélé: investigations and contact tracing continue around the confirmed case in the Agri-Uélé health area (Ganga Health Zone).

1.1.4. Challenges

Poor contact tracing around confirmed cases mainly in the Bas Uélé province due to insufficient teams dedicated to these activities.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

- Five (5) alerts were notified to the PoE/PoC in the last 24 hours: three in Ituri, including two confirmed live alerts transferred to the CT/CTE (100%) and one body intercepted at the Base Yedi PoC (Komanda Health Zone), which tested positive for COVID-19; two in North Kivu, consisting of two bodies intercepted at the Kasitu PoC (Beni Health Zone) and sent to the Beni General Hospital morgue for testing and securing the remains. No alerts were notified in Haut-Uélé, Bas-Uélé, Tshopo, or South Kivu.

Indicators	Ituri	North-Kivu	South-Kivu	Tshopo	Haut-Uele	Down-Uele	Overall
Report completeness	98.3% (59/60)	100% (18/18)	ND	71.4% (5/7)	54.5% (6/11)	35.0% (7/20)	81.9% (95/116)
% of PoE/PoC strategic functions	9.09% (1/11)	100% NA		35% 55%	(6/11)	ND	ND
Number of people who have switched to PoE/PoC	138,871	83,918	84 432 8 561		8,167	ND 323 949	
% of travelers screened	97.0% 97.89%	100% 99.3%	100% 100%	98.1%			
% of travelers who washed their hands	96.8% 97.89%	100% 99.1%	100% 100%	98.1%			
% of travelers made aware	99.4% 97.89%	100% 100%	99.6% ND				99.2%
Number of alerts notified	3	2	0	0	0	0	5
Number of alerts validated live cases (suspected cases)	2	0	0	0	0	0	2

Indicators	Ituri	North-Kivu	South-Kivu	Tshopo Haut-Uele	Down-Uele	Overall
Number of bodies recovered today	1	2	0	0	0	3
% of suspected cases transferred to CT/CTE	100% (2/2)	N / A	0% 0%		0%	0% 100% (2/2)
% of lifeless bodies swabbed/secured	100% (1/1)	0% (0/2)	0% 0%		0%	0% 33.3% (1/3)
% of suspected cases investigated within 24 hours	100% (2/2)	100% 0% 0%			0%	0% 100% (2/2)

1.2.2. Challenges

Insufficient operationalization of PoE/PoC projects: national completeness stands at 81.9% (95 out of 116 reports), with low levels in Bas-Uélé (35.0%) and Haut-Uélé (54.5%). In Ituri, only 1 (9.1%) of the 11 strategic PoE/PoC projects is operating without interruption; in Haut-Uélé, five PoC projects (Dusu, Apodo, Aungba, PK6 Niangara, and Gossamu) did not operate due to a strike by unpaid service providers; Tshopo shows partial reporting completeness (5/7 PoE/PoC projects). In South Kivu and Haut-Uélé, demotivation among stakeholders due to non-payment persists and has led to the suspension of five PoC projects in Haut-Uélé.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 34 new positive results** (34 living and 0 deaths) out of 231 new samples received and analyzed (**positivity: 14.7 %**).
- **North Kivu : 18 new positive results** (9 living and 9 dead) out of 178 samples analyzed (**positivity of 10.1 %**).
- **Haut-Uélé: 5 new positive results** (3 living and 2 dead) out of 18 samples analyzed (**positivity of 27.8%**).
- **Bas-Uélé : no new positive results on 3 samples analyzed (positivity 0%).**

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 21 of the 101 identified infection control rings were opened (20.8%), 165 of the 173 identified households were decontaminated (95.3%), and all 11 health facilities (100%) were decontaminated; 6 health facility assessments were conducted, and 17 infection prevention and control (IPC) kits were distributed to households. The health data collection (HDC) pillar received 86 alerts (10 reports): 71 bodies were swabbed, and 82 HDCs were carried out.
- **In North Kivu**, the EDS pillar received 37 alerts (15 reports): 36 bodies were found, a failure of swab (refusal of the body bag in Kalunguta) and 34 EDS carried out.
- **In Bas-Uélé**, 3 rings were opened (Buta, Viadana and Ganga), 2 households were decontaminated in Ganga and the Buta General Referral Hospital was assessed (card score of 54.3%). Two death alerts were received and 2 bodies were swabbed.
- **In Tshopo**, 10 ESSs were supported in 3 health zones (Makiso-Kisangani, Mangobo and Tshopo), where 106 service providers were briefed; followed by the functionality of 10

handwashing stations and 10 triage units installed; training of 35 hygienists (5 teams) from 4 health zones was carried out.

1.4.2. Challenges

- Weak implementation of PCI standards: in Ituri, ring opening remains limited to 20.8 % and swab failures were recorded; one member of the EDS team died in a traffic accident (ZS Bambu) and two others were attacked in Rwampara (AS Makabo).
- In Bas-Uélé, the decontamination of the household of relatives of the first confirmed case in Buta has still not been able to be carried out due to continued community resistance, the pre-breakup of several PCI inputs delaying the briefing of the service providers in the ZS.

1.5. Continuity of care

1.5.1. Main actions

- In Ituri, 88 new admissions were recorded and 97 discharges, including 20 recoveries. Thus, 488 patients are hospitalized out of 978 beds, representing an overall occupancy rate of 49.9%.
- In North Kivu, 53 admissions were recorded compared to 37 discharges, including 1 recovery. At the end of the day, 194 patients were hospitalized in 206 beds, representing an overall occupancy rate of 94.2%.
- In Tshopo, there were 3 new admissions of suspected cases and 8 patients were isolated out of 14 beds, representing 57.1% occupancy.
- In Bas Uélé, 1 confirmed case is in isolation at HGR Ganga.
- In Haut Uélé, 9 admissions were recorded compared to 6 discharges, including 3 recoveries. A total of 79 patients are in isolation out of 120 beds, representing an occupancy rate of 65.8%.

1.5.2. Challenges

Low reception capacity and absence of CTEs in some health zones; saturation of the Bambu (confirmed) and Kilo (suspected) CTEs in Ituri and exceeding capacity in suspected beds in North Kivu (163 patients for 142 beds) and in Haut-Uélé (Wamba CTE in oversaturation in suspected beds).

1.6. Risk communication and community engagement

1.6.1. Main actions

- In Ituri, 335 contacts were identified around confirmed cases (211 in Rwampara and 124 in Lolwa); 267 people were sensitized in the community and 43 school inspectors, including 8 women, were trained in Bunia
- In Tshopo, 1,116 people were sensitized during home visits in 186 households in Makiso, Mangobo and Kabondo; 3 shows were hosted and spots broadcast on 6 radio and television channels, 61 religious leaders (52 men and 9 women) were sensitized and a rumor was clarified.
- In North Kivu, 44 people were affected around the confirmed cases in the health areas of Madrandele (40, including 8 providers) and Mabolio (4); data on clarified rumors and mass awareness campaigns are not available for the day.
- In Haut-Uélé, 14 PCI kits were provided to social and solidarity economy (SSE) organizations, 8 assessments and 11 SSE support sessions were conducted, and 55 service providers were briefed; 7 rings were opened (Wamba 4 and Pawa 3), 6

Households and 3 ESS decontaminated. The EDS pillar received 7 alerts, 7 bodies were successfully swabbed and 5 EDS carried out.

- In **Bas-Uélé**, awareness-raising activities on Ebola virus disease, prevention measures and dignified and safe burials were organized for the youth group on Mobenge Avenue in Buta, along with the distribution of a handwashing station.

1.6.2. Challenges

Persistent challenges at the community level: in Ituri, refusals to provide samples (Nia-Nia), to participate in swab testing (Alimasi), and to register contacts (Kilo État). In Haut-Uélé, response teams were attacked in Isiro and Pawa.

1.7. Mental Health and Psychosocial Support (MHPSS)

1.7.1. Main actions

- In **Ituri**, 25 new confirmed cases and 107 confirmed cases under follow-up benefited from SMSPS interventions, as well as 34 new suspected cases and 119 suspected cases under follow-up; 51 households received support, 15 people living with HIV were assisted, 60 children received community follow-up, and 70 test results were announced, 25 of which were positive. Twelve out of 20 recovered patients (60%) have been reintegrated into the community.
- In **North Kivu**, 11 of the 18 new confirmed cases (61.1%) and all of the 57 new suspected cases (100%) benefited from SMSPS interventions; 68 results were announced, including 11 positive cases, 72 PPLs were accompanied and 22 separated or orphaned children were followed in the community.
- In **Haut-Uélé**, 1 new confirmed case and 22 confirmed cases under follow-up received psychosocial support, as well as 3 new suspects, 33 accompanying persons and 15 PPL; 17 announcements of results were made, including 2 positive ones.
- In **Tshopo**, 4 new suspected cases and 4 suspected cases under follow-up received an interview and psychological support, as well as a companion of a hospitalized case; 4 laboratory results were announced, none of which were positive, and 27 contacts received psychological support.

1.7.2. Challenges

In **Haut Uélé**, coverage remains limited to 2 of the 6 affected health zones (33.3%), due to the insufficient number of psychosocial agents.

1.8. Logistics

1.8.1. Main actions

- Continued provision of mobility for field actors in the various provinces and delivery of IPC inputs to affected health zones.

1.9. Security

1.9.1. Main actions

- Supporting teams deployed across different areas by conducting security monitoring in hard-to-reach health zones.

1.9.2. Challenges

The securing of strategic PoCs remains incomplete: a lifeless body intercepted at the Mangina PoC (ZS Komanda) could not be swabbed due to security concerns.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions

Continued support for training of community, military and police leaders on PSEA and MVE, jointly with the communications pillar.

1.10.2. Challenges

- Still insufficient coverage of PSEA activities, limited to 4 out of 36 health zones in Ituri, lack of mapping of stakeholders, insufficient mass awareness materials and lack of transport and computer equipment for the pillar; Coverage remains limited to 4 of the 36 health zones of Ituri (11.1%) and only two partners (WHO and Africa CDC) report their PSEA activities.

KEY MESSAGES

- Ebola virus transmission remains active, with 57 new cases including 11 community deaths and 18 intra-CTE recorded in the last 24 hours, bringing the national total to 5,656 confirmed cases and 2,715 deaths.
- Ituri remains the epicenter of the epidemic, concentrating 83.1% of cumulative confirmed cases and 59.6% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).
- Surveillance shows good performance (100.0%) of suspected cases investigated while the follow-up rate (84.6%) is close to the threshold of 85.0%.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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